

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewed by:	[Signature]	
Date Reviewed:	5/27/11	

MRN: [REDACTED]
Surgery [REDACTED]

MD

OV

Hospital [REDACTED]

UUID: 3E54B0CE-8863-4728-BC95-A47C1869F52A
TCGA-AC-A2BK-01A-PR

Redacted

Laboratory Patient Report
Print Date/Time:



Surgical Pathology

Histopathological Examination

Path# [REDACTED]
Collected: [REDACTED]

Received: [REDACTED]

Complete

Pre-Op Diagnosis : Left Breast Ca
Order Physician : [REDACTED] MD
Specimens : Breast, left
Frozen Diagnosis :

1CB-0-3
carcinoma, infiltrating duct, NOS 8500/3
Site: breast, NOS C50.9 w/ 5/27/11

Report : GROSS EXAMINATION:

The specimen is received in a container labeled with the name of the patient and labeled as breast, left. The specimen consists of a mastectomy designated left which measures en bloc 31 x 20 x 5.5 cm and weighs 1515 g. Anteriorly located is an ellipse of brown skin which measures 22 x 17 cm. Centrally located is a 4.5 cm diameter areolar complex with slightly retracted nipple which is sampled in block 1. The deep margin is inked and the specimen is serially sectioned and shows a gray-tan firm mass in the upper outer quadrant. This mass measures 2.8 x 2.5 x 2.3 cm and grossly extends to within 1 cm of the inked deep margin. A section of skin overlying the mass is submitted in block 2. A section of the inked deep margin is submitted in block 3. Sections of the mass are submitted in block 4-7. The remaining breast is composed of yellow lobulated adipose tissue admixed with islands of gray-white rubbery tissue. The breast is divided into four quadrants; upper inner, lower inner, upper outer and lower outer and sampled in blocks 8-11. The attached axillary tissue is dissected for lymph nodes with the lower part sampled in blocks 12-14 in the upper part sampled in blocks 15-16.

DIAGNOSIS BASED ON GROSS AND MICROSCOPIC EXAMINATION:

SPECIMEN TYPE: Modified radical mastectomy.
LYMPH NODE SAMPLING: Axillary dissection.
LATERALITY: Left
TUMOR SITE: Upper outer quadrant
SIZE OF INVASIVE COMPONENT: Greatest dimension 2.8 cm.
ADDITIONAL DIMENSIONS: 2.5 x 2.3 cm.
HISTOLOGIC TYPE: Invasive ductal carcinoma.

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MR#: [REDACTED]

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HISTOLOGIC GRADE: Nottingham histologic score 3/3.
DUCT IN SITU COMPONENT: Not identified.
MICROCALCIFICATIONS: Not identified.
EXTENT OF INVASION
PRIMARY TUMOR (PT): pT2: Tumor more than 2.0 cm but not more than 5.0 cm in greatest dimension.
REGIONAL LYMPH NODES (PN): pN2a: Metastases in 4 to 9 axillary lymph nodes.
NUMBER EXAMINED: 23
NUMBER INVOLVED: 6
DISTANT METASTASIS (PM): PMX
MARGINS: Uninvolved by invasive carcinoma.
DISTANCE TO NEAREST MARGIN: 1.0 cm to deep margin.
VENOUS/LYMPHATIC (LARGE/SMALL VESSEL) INVASION (V/L): Present.

Summary of pathologic staging: pT2N2aMXG3.
Intradepartmental consultation obtained.

COMMENT: Immunohistochemical stains for estrogen and progesterone receptors and HER2 are pending and will be supplementally reported when complete.

[REDACTED] M.D.
Pathologist

Electronically Signed by:
[REDACTED], MD

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